Pembrolizumab + carboplatin/paclitaxel → AC

> **Confirm the individual supportive prescription.** This review aid does not generate a medication order. Use the current NCCP regimen, antiemetic guidance and local pharmacy plan; assess interactions, allergies and comorbidities.

Antiemetic and premedication review

- Phase-dependent: high Day 1 carboplatin/paclitaxel and AC; low weekly paclitaxel-only days; minimal pembrolizumab alone
- Premedicines, anti-sickness medicines and growth-factor plans may change when paclitaxel/carbo changes to AC.
- Report new diarrhoea, breathlessness, rash, headache or marked fatigue, including after treatment ends.

Additional supportive care

- Premedicines, anti-sickness medicines and growth-factor plans may change when paclitaxel/carbo changes to AC.
- Report new diarrhoea, breathlessness, rash, headache or marked fatigue, including after treatment ends.
- Weekly visits in phase 1; every-three-week AC visits in phase 2. There is no routine home chemotherapy pump.
- Discuss fertility preservation before treatment and ask about scalp cooling, exercise and practical support.

Monitoring and handover

- Know when the chemotherapy changes: After four 3-week cycles. This is q3-week AC 60/600. Do not substitute a dose-dense AC or pembrolizumab 400 mg neoadjuvant variant.
- Thyroid and other immune checks: Thyroid tests every 3–6 weeks; glucose each cycle; other tests as indicated. NCCP: cortisol as indicated. Grade-specific immune toxicity is handled separately from cytotoxic dose changes. Pembrolizumab is withheld/stopped, not dose-reduced.
- Heart assessment around doxorubicin: Before and during the anthracycline phase, according to risk. NCCP specifies baseline ECG and echo/MUGA when >65 or clinically indicated, with LVEF >50% for doxorubicin. ESC recommends broader baseline echocardiographic assessment for anthracyclines and risk-based follow-up. Agree the local cardiac pathway.

- Postoperative medicines and radiotherapy: After pathology and recovery. Residual-disease or germline-BRCA-directed additions require separate indication review; this map does not automatically add capecitabine or olaparib.
- Surveillance and lasting effects: After early breast treatment. Routine CT, PET or tumour-marker blood tests are not recommended simply to look for recurrence after early breast treatment in someone without symptoms. Imaging is arranged for a clinical reason. DEXA is not routinely required solely because of pembrolizumab; assess if treatment causes menopause or another bone-risk indication arises.

Access, visits and patient preparation

- Port insertion and removal: The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.
- PICC care and urgent line symptoms: A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.
- Visit time and transport: First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Patient handover

Explain the current phase, how to take each medicine, which tests are expected next and who arranges and reviews them. Confirm urgent contact details. Provide the patient portal, visual guide, passport, treatment card and whole-treatment map.

Fever, shivers or suddenly feeling unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: emergency help, 112 / 999.

Sources

NCCP 00857 v3 · official regimen

ESC 2022 cardio-oncology guideline · cardiac surveillance and risk

NCCP supportive-care antiemetics for NCIS